

Lifelong Opportunity Guide 81 — Health Information Technologist and Medical Registrar

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Language: English

Primary U.S. benchmark: O*NET-SOC 29-9021.00 — Health Information Technologists and Medical Registrars

Canada comparison: NOC 12111 — Health information management occupations

Colombia comparison: CUOC 32520 — Técnicos en documentación sanitaria

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What this career is

Health information technologists and medical registrars turn clinical and administrative health information into accurate, secure and usable records, classifications, registries, reports and information systems. Depending on the role, they may abstract information from medical records, classify or code data, support electronic health record systems, maintain disease or treatment registries, monitor data quality, prepare reports, retrieve records, support privacy/security procedures, educate staff and help improve health-information workflows.

The work sits between healthcare, information management and technology. It requires enough medical terminology and healthcare-process knowledge to understand documentation, enough information-management skill to preserve accurate records, and enough technical judgment to recognize when a coding, privacy, clinical or system issue must be escalated.

This guide uses **O*NET-SOC 29-9021.00 — Health Information Technologists and Medical Registrars** as its primary United States benchmark. Canada maps closely to **NOC 12111 — Health information management occupations**. Colombia has a direct occupational match in **CUOC 32520 — Técnicos en documentación sanitaria**.

Why this can be a strong opportunity

The U.S. occupation is currently classified as Bright Outlook. Health organizations continue digitizing records, improving data quality, using registries and analytics, expanding interoperability, managing privacy/security requirements and introducing automation. Those trends increase the importance of people who understand both health information and systems.

The role can be a pathway from healthcare administration, medical records, coding, billing, registration, clinical support, data entry, quality, public health, insurance, records management, information technology or customer/patient services.

A practical career ladder can look like:

health administration or records support → health-information technician/registrar → coding/registry/data-quality specialization → health-information analyst, systems specialist, clinical documentation, informatics, supervisory or management work.

The exact path depends on education, jurisdiction, employer, specialty and certification requirements.

Health information technologist, medical registrar, coder, medical records specialist and health informatics specialist are not identical roles

Health information technologist

May work with:

- EHR and health-information systems;
- data quality and validation;
- reporting;
- databases and data extracts;
- system implementation or upgrades;
- workflow analysis;
- privacy/security controls;
- user education;
- records indexes and retrieval systems.

Medical registrar

“Registrar” in this context can refer to disease, tumor/cancer or other specialized clinical registries—not a physician training title. A registrar may:

- abstract defined clinical information;
- classify cases under registry rules;
- track follow-up;
- maintain registry completeness and quality;
- prepare statistics/reports;
- support research, surveillance or quality improvement.

Specialty registries may require specific training or credentials.

Medical coder

A coder applies approved coding systems and rules to documented clinical information. Coding is not diagnosis. A coder should not invent a diagnosis because it seems medically plausible.

Medical records specialist

May focus more heavily on record completeness, release of information, scanning/indexing, document queues, record correction workflows, retrieval, patient identity, filing and administrative record controls.

Health informatics specialist

May have broader responsibility for clinical systems, workflow design, data standards, analytics, interoperability, implementation and technology strategy. Some roles require more advanced education or technical experience.

Professional and clinical boundaries

A health-information title does **not** automatically authorize a worker to:

- diagnose a disease;
- prescribe or recommend treatment;
- independently interpret a clinical finding for patient care;
- invent or infer a diagnosis so a claim or registry record can be completed;
- change a clinician's source documentation outside an approved amendment/correction workflow;
- assign a code unsupported by the documentation and applicable rules;
- remove clinically relevant information because it appears inconvenient;
- disclose protected health information simply because technical access exists;
- independently determine legal or regulatory compliance unless that responsibility is formally assigned;
- determine retention/destruction schedules or remove a legal hold independently;
- design production cybersecurity architecture without assigned authority;
- use real protected health information in a public portfolio or unapproved AI system;
- claim a credential, certification, licence, registration or accreditation not actually held.

When documentation is incomplete, conflicting or unclear, use the approved clinician/coder/registry query or escalation process. O*NET explicitly recognizes consultation with physicians or others when codes or diagnoses are conflicting, missing or unclear.

The health-information lifecycle

1. Capture the source information

Health information can originate from:

- clinician documentation;
- diagnostic reports;
- laboratory/imaging results;
- orders;
- operative/procedure notes;
- medication records;
- admission/discharge data;
- registration/demographic data;
- billing/claims data;
- device/system feeds;
- public-health reporting;
- patient-reported information;
- external health records.

The worker must know which source is authoritative for the task and whether the information is complete enough to proceed.

2. Confirm patient and record identity

Patient matching errors can create serious clinical and privacy risks. Follow approved identity procedures and pay attention to:

- name and aliases;
- date of birth;
- medical record number;
- encounter/account number;
- address/contact information;
- duplicate records;
- merged/unmerged identities;
- facility/location;
- dates of service.

Do not merge or alter identities outside assigned authority.

3. Abstract and classify

Abstraction means extracting defined information from the record according to a data dictionary, registry rule, coding rule or reporting requirement.

Good abstraction requires:

- consistent definitions;
- source traceability;
- correct date/time context;
- distinguishing documented fact from interpretation;
- handling missing/unknown values correctly;
- resolving ambiguity through approved query/escalation.

4. Validate data quality

Health-information quality commonly includes:

- completeness;
- accuracy;
- consistency;
- timeliness;
- validity;
- uniqueness;
- correct patient identity;
- correct coding/classification;
- traceability to source;
- appropriate access/security status.

A populated field is not necessarily a correct field.

5. Store and protect

Follow approved rules for:

- EHR/database storage;
- document indexing;
- record status;
- version/amendment history;
- permissions;
- retention;
- backup/recovery processes;
- legal hold;
- audit logging;
- release of information.

6. Retrieve and release

Before releasing information, verify:

- requester identity;
- authority/authorization;
- applicable release-of-information workflow;
- correct patient;
- correct date range/document type;
- minimum necessary scope where applicable;
- redaction requirements;
- secure delivery method;
- logging/documentation requirements.

7. Analyze and report

Health-information staff may prepare:

- census/utilization reports;
- registry reports;
- quality metrics;
- coding or documentation trends;
- completeness reports;
- operational dashboards;
- disease/treatment statistics;
- data-quality exception reports.

Know the difference between presenting data and making a clinical conclusion.

Clinical coding and documentation integrity

Coding work may use diagnostic/procedure classification systems, DRG/grouping software and specialized encoder tools. Exact systems and rules vary by country, payer, specialty and setting.

Core rules:

- code from authorized documentation;
- apply the required coding standard/version;
- do not create missing diagnoses;
- preserve documentation/coding auditability;
- query unclear or conflicting information under approved procedures;
- distinguish clinical validation from coding validation;
- do not change provider documentation merely to obtain a preferred reimbursement result;

- escalate suspected fraud or inappropriate pressure through authorized channels.

Medical registry work

Registries may track cancer/tumors, trauma, implants, diseases, procedures, outcomes or other defined populations.

A strong registrar understands:

- inclusion/exclusion criteria;
- case finding;
- abstraction rules;
- follow-up requirements;
- staging/classification rules where applicable;
- required source documents;
- quality edits;
- duplicate prevention;
- report deadlines;
- confidentiality;
- registry-specific governance.

Never apply a clinical stage/classification outside the rules and training required by that registry.

Health-information systems

Useful concepts include:

- EHR/EMR;
- master patient index;
- clinical documentation systems;
- laboratory/radiology interfaces;
- coding/encoder tools;
- release-of-information systems;
- document imaging;
- registry software;
- database/query tools;
- reporting/business intelligence;
- interoperability concepts;
- identity/access management;
- audit logs;
- downtime/recovery procedures.

A technologist may support system testing, implementation, training and improvement. That does not automatically make the person a database administrator, software engineer or cybersecurity architect.

Data quality: use a repeatable method

A practical review cycle can be:

1. identify the data element and authoritative source;
2. verify patient/encounter identity;
3. apply the current definition/rule;
4. capture or validate the value;
5. run required edits;
6. review missing/conflicting information;
7. query/escalate when needed;
8. document resolution;
9. confirm downstream report/registry/system impact;
10. preserve audit evidence.

Do not “clean” clinical data by removing unusual but legitimate values.

Privacy and confidentiality

Health information is sensitive. Good practices include:

- access only records needed for assigned work;
- never use someone else's account;
- protect passwords and MFA;
- lock the workstation when away;
- verify recipients before release;
- avoid discussing records in public spaces;
- use approved storage and transmission tools;
- report misdirected information promptly;
- follow printing/shredding/paper-record rules;
- avoid unnecessary downloads or exports;
- do not browse records out of curiosity.

Privacy law varies by jurisdiction. Follow applicable law and employer policy; this guide does not provide legal advice.

Cybersecurity in health-information work

Health systems are frequent targets of phishing, credential theft, ransomware and data exfiltration. Practical controls include:

- recognize suspicious emails/messages;

- use MFA where supported;
- report unexpected MFA prompts;
- verify unusual requests for large exports;
- use approved devices and networks;
- never disable security tools to make work faster;
- avoid unapproved USB drives or personal cloud services;
- follow downtime procedures during outages or cyber incidents;
- report suspected unauthorized access quickly.

A successful login or export does not prove the action is authorized.

Responsible AI and automation

Health-information workflows may use OCR, computer-assisted coding, NLP, rules engines, duplicate-detection tools, predictive models and generative AI.

Approved automation can help with:

- extracting candidate fields;
- identifying missing documentation;
- suggesting possible classifications;
- finding duplicates;
- generating synthetic training data;
- drafting generic training explanations;
- summarizing non-sensitive policy material.

Controls for real health information:

- use only approved systems and approved data classes;
- preserve source traceability;
- verify extracted/coded/summarized output;
- never allow AI to invent a diagnosis or missing clinical fact;
- require human approval where policy requires it;
- monitor systematic error and bias;
- protect protected health information;
- preserve audit logs;
- escalate unsafe or inexplicable outputs.

NIST's AI Risk Management Framework is voluntary risk-management guidance. The Generative AI Profile addresses generative-AI-specific risks. Neither replaces clinical, coding, privacy, security or legal governance.

Accessibility and inclusive health-information systems

Health-information systems and documents should support users with disabilities where required. Useful principles include:

- keyboard access;
- meaningful field labels;
- readable contrast and text;
- visible focus;
- error messages that explain how to correct a problem;
- logical form order;
- accessible electronic documents;
- assistive-technology compatibility;
- captions/transcripts for training media where appropriate;
- avoiding color-only meaning.

An automated accessibility checker cannot prove complete legal accessibility conformance.

United States pathway

Preparation

O*NET places the occupation in **Job Zone Three — Medium Preparation Needed**. Most occupations in this zone typically require vocational training, related experience or an associate degree, with one to two years of on-the-job/informal training common.

Potential entry backgrounds include:

- health information technology;
- medical records;
- medical coding;
- healthcare administration;
- registry work;
- health data/analytics;
- clinical support plus information-system experience;
- IT support within healthcare.

Registered Apprenticeship

O*NET lists example approved apprenticeship titles:

- Clinical Documentation Improvement Specialist;
- Health Information Technologist (Nof);
- Tumor Registrar.

An approved title does not mean an apprenticeship opening is currently available in every area.

WIOA and training support

CareerOneStop can help locate WIOA-eligible training. Confirm individual eligibility, credential recognition and employer requirements before enrolling.

United States wages and outlook

O*NET/BLS 2025 national wage data for **29-9021.00**:

Percentile	Annual	Hourly
10th	\$39,830	\$19.15
25th	\$48,940	\$23.53
Median	\$68,020	\$32.70
75th	\$95,630	\$45.98
90th	\$117,420	\$56.45

2024-2034 projections:

- employment 2024: **41,900**;
- projected employment 2034: **48,100**;
- projected growth: **15%**;
- projected **annual openings**: **3,200**.

Current non-government related-title context

Indeed reported approximately **\$21.15/hour** for **Health Information Technician**, based on **701 salaries** from job postings in the prior 36 months, updated **July 31, 2026**.

This is a related-title market estimate and can represent a more junior/narrower population than the official O*NET occupation. Do not substitute it for the official wage series.

Current U.S. software signals

O*NET employer-posting data for 2025 show:

- Microsoft Office **12%**;
- Microsoft Excel **8%**;
- Epic Systems **4%**;
- Microsoft Outlook **4%**;
- Microsoft Word **2%**;
- Microsoft PowerPoint **2%**;

- MEDITECH **1%**;
- Microsoft Access **1%**.

O*NET also identifies EHR/medical software, coding tools, SQL/database/reporting tools, document management, OCR/scanning and analytics software. Learn transferable concepts rather than assuming one product is universal.

Canada pathway

Canada Job Bank identifies **NOC 12111 — Health information management occupations**.

Current Job Bank requirements state that health information management technicians and related workers require completion of a **recognized two-year college diploma** in health record technology or health information management.

Workers are eligible to write the **Canadian Health Information Management Association national certification examination**, which **may be required by employers**.

Current Job Bank records indicate the occupation is **not regulated in Canada**. “Not regulated” does not mean employers cannot require certification.

National wages updated November 19, 2025:

- low: **C\$22.00/hour**;
- median: **C\$30.51/hour**;
- high: **C\$46.19/hour**.

Canada.ca can help locate recognized training, student aid and skills resources. Verify whether a particular program satisfies employer/certification expectations.

Colombia pathway

CUOC 32520 — Técnicos en documentación sanitaria

This is a direct occupational match. Relevant titles include:

- Administrador de información de salud;
- Administrador de registros de salud;
- Analista de registros médicos;
- Auxiliar de codificación clínica;
- Auxiliar de información de salud;
- Auxiliar de registros médicos;
- Supervisor de auxiliares de registros en salud;

- Supervisor de unidad de registros médicos;
- Técnico de información de salud;
- Técnico de información sanitaria;
- Técnico de registro de enfermedades;
- Técnico de registros médicos.

Functions include managing health-record indexes/storage/retrieval, processing medical/admission records, supporting epidemiological/research/billing/cost/care-improvement information and reviewing records for completeness/accuracy/compliance.

OCUPACOL does not provide a statistically representative wage series suitable for a controlled national salary claim for this guide. Do not invent one.

SENA — Apoyo administrativo en salud

SENA Betowa lists **Apoyo administrativo en salud** as a **Técnico** program of **2,640 hours**.

Relevant competencies include admitting users under health rules, processing affiliations, registering information according to regulations/technical procedure and using information tools.

This is a useful foundation, not a claim of equivalence to every HIM, coding or registry credential. Cohorts, locations and seats change.

Latin America and Caribbean pathway

OIT/Cinterfor's country/institution observatory can help identify national vocational-training institutions. Verify the current program, credential, admission, professional recognition, modality and cost directly with the institution.

Synthetic portfolio: never use real patient data

A portfolio can demonstrate skill without exposing protected information.

Use fully synthetic or public non-identifiable data for examples such as:

1. a synthetic health-record completeness audit;
2. a mock registry data dictionary;
3. a synthetic duplicate-patient identification exercise;
4. a coding/documentation query decision flow;
5. a data-quality dashboard;
6. a release-of-information checklist;
7. a mock EHR workflow map;
8. an access-control matrix;

9. a synthetic registry follow-up queue;
10. a privacy/security incident decision tree.

Do **not** use real patient names, medical record numbers, real dates of service tied to individuals, screenshots from production EHR systems or any employer/client protected data unless disclosure is explicitly authorized and lawful.

Resume evidence

Strong evidence-based bullets may describe:

- record completeness improvement;
- coding or registry quality work;
- data validation;
- patient-identity cleanup;
- backlog reduction;
- EHR implementation/support;
- report/dashboard production;
- user training;
- release-of-information workflow;
- privacy/security compliance;
- audit support;
- system testing;
- query resolution.

Use only real metrics you can support.

Interview preparation

Expect questions such as:

- What do you do if documentation does not support the requested code?
- How do you resolve conflicting clinical documentation?
- How do you protect sensitive health information?
- What makes health data high quality?
- How would you validate a registry abstraction?
- How do you prevent duplicate patient records?
- What is your experience with EHR systems?
- How would you respond to an unusual request for a large data export?
- How would you validate AI-assisted coding or extraction?
- How do you distinguish coding from clinical diagnosis?

A strong answer emphasizes source evidence, applicable rules, verification and escalation.

Questions to ask an employer

Ask:

- Which EHR/registry/coding systems are used?
- What credentials are required versus preferred?
- Which coding/registry standards apply?
- How are clinician queries handled?
- What are the data-quality metrics?
- Who owns privacy and release-of-information decisions?
- What access-control model is used?
- What are the downtime/cyber-incident procedures?
- Which AI or computer-assisted coding tools are approved?
- What human review is required for automation?
- What training is provided?
- What advancement paths exist toward analyst, informatics, registry, CDI or management roles?

First 30 days

Days 1-5

- learn the organization's record and privacy policies;
- understand patient-identity workflow;
- learn EHR/registry/coding systems;
- identify authoritative source documentation;
- learn escalation/query channels;
- understand access and release controls.

Days 6-15

- complete supervised abstractions or record reviews;
- learn data-quality edits;
- practice record retrieval and release workflow;
- review duplicate-patient controls;
- learn audit/history functions;
- study the applicable coding/registry standards.

Days 16-30

- improve accuracy and turnaround time;
- identify recurring documentation/data-quality issues;
- learn one reporting or query tool;
- participate in supervised system/quality review;
- request feedback using actual quality metrics.

90-day skill-building plan

Month 1 — health records and privacy

Build medical terminology, record-lifecycle, confidentiality, patient identity and EHR fundamentals.

Month 2 — classification, registry and data quality

Learn relevant coding/registry rules, abstraction, edits, queries, data dictionaries, completeness and quality measurement.

Month 3 — systems and analytics

Add SQL/query concepts, reporting, dashboards, system testing, workflow mapping, interoperability concepts, AI governance and privacy/security awareness.

Practical health-information checklist

Before processing:

- ☐ Correct patient/encounter identified.
- ☐ Authoritative source confirmed.
- ☐ Required documentation present.
- ☐ Correct standard/version/work queue selected.

During processing:

- ☐ No unsupported clinical inference.
- ☐ Coding/abstraction rules applied.
- ☐ Missing/conflicting information queried or escalated.
- ☐ Source traceability retained.
- ☐ Confidentiality maintained.

Before completion:

- ☐ Required edits passed/reviewed.
- ☐ Record/registry status correct.
- ☐ Audit/query evidence recorded.
- ☐ Required report/release routing completed.

Practical privacy/release checklist

- ☐ Requester identity verified.
- ☐ Authorization/authority confirmed.
- ☐ Correct patient and date range confirmed.
- ☐ Minimum necessary/approved scope applied.
- ☐ Redaction reviewed where required.

- ☐ Secure channel used.
- ☐ Release logged/documented.
- ☐ No unauthorized copy retained.

Practical AI/automation checklist

- ☐ Tool is approved for the data class.
- ☐ Protected information is handled only as authorized.
- ☐ Source traceability is preserved.
- ☐ AI/OCR/coding output is verified.
- ☐ Missing clinical facts are not invented.
- ☐ Required human approval is complete.
- ☐ Systematic errors/exceptions are reviewed.
- ☐ Audit evidence is retained.

Sources to verify before important decisions

- O*NET occupation details: <https://www.onetonline.org/link/details/29-9021.00>
- O*NET summary: <https://www.onetonline.org/link/summary/29-9021.00>
- O*NET Job Zone: <https://www.onetonline.org/skills/zone/29-9021.00>
- O*NET/BLS wages: <https://www.onetonline.org/link/localwages/29-9021.00>
- O*NET/BLS trends: <https://www.onetonline.org/link/localtrends/29-9021.00>
- O*NET software demand: <https://www.onetonline.org/link/demand/29-9021.00>
- O*NET hot technologies: https://www.onetonline.org/link/hot_tech/29-9021.00
- CareerOneStop WIOA: <https://www.careeronestop.org/LocalHelp/EmploymentAndTraining/find-WIOA-training-programs.aspx>
- Indeed Health Information Technician: <https://www.indeed.com/career/health-information-technician/salaries>
- Canada Job Bank summary: <https://www.jobbank.gc.ca/marketreport/summary-occupation/25618/ca>
- Canada Job Bank requirements: <https://www.jobbank.gc.ca/marketreport/requirements/25618/ca>
- Canada Job Bank wages: <https://www.jobbank.gc.ca/wagereport/occupation/25618>
- Canada training gateway: <https://www.canada.ca/en/services/jobs/training.html>

- Colombia OCUPACOL CUOC 32520: <https://ocupacol.mintrabajo.gov.co/Profile/OccupationalProfile/32520>
- SENA Apoyo administrativo en salud: <https://betowa.sena.edu.co/oferta/apoyo-administrativo-en-salud?location=57011001&modality=P&offertype=open&programId=212936>
- OIT/Cinterfor: <https://www.oitcinterfor.org/statsfp/paises>
- CISA Secure Our World: <https://www.cisa.gov/secure-our-world>
- NIST AI Risk Management Framework: <https://www.nist.gov/itl/ai-risk-management-framework>
- NIST Generative AI Profile: <https://www.nist.gov/publications/artificial-intelligence-risk-management-framework-generative-artificial-intelligence>
- Section508.gov: <https://www.section508.gov/create/>

Assurance boundary

This guide provides general educational and career-planning information. It is not medical, coding, legal, privacy, cybersecurity, records-retention, financial, employment, immigration or regulatory advice. It does not certify clinical competence, coding accuracy, professional registration, licensure, privacy compliance, accessibility, cybersecurity or translation quality.

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